



Fiona Stanley Fremantle Hospitals Group

Guideline

Toxicology patient management: Emergency Department/Intensive Care/Emergency Short Stay Unit

Reference #: FSH-ED-GUI-0022

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital (FSH)	Emergency Department (ED) Intensive Care Unit (ICU) Emergency Short Stay Unit (ESSU)	Medical, Nursing, Allied Health

1. Introduction

This guideline outlines a cohesive, collaborative, multidisciplinary team approach to the management of toxicology presentations at Fiona Stanley Hospital which require an Intensive Care Unit (ICU) admission. The ED has a high turnover of patients in the Emergency Short Stay Unit (ESSU), which enables appropriate toxicology patients to be decanted to ED post extubation. This cohort of patients previously had a prolonged stay in ICU awaiting an available general medicine bed.

2. Terminology

ANUM	Associate Nurse Unit Manager. Working 0700-2330hrs Monday - Sunday
ACEM	Australasian College of Emergency Medicine
Medical Clearance	Term used to indicate to all treating teams that patient is suitable from a physical health perspective to be admitted to a psychiatric facility. This DOES NOT mean that there are no ongoing medical needs only that they no longer require an acute ED bed and that an appropriate ongoing plan including medications and observations required is documented

3. Team structure and service availability

- The ED toxicology team currently comprises of an ACEM accredited Special Skills Post Registrar supervised by two ED Physician and Clinical Toxicologists. This team operates in conjunction with the Royal Perth Toxicology service now known as the Southeast Toxicology Service (SETS)
- Consultation service is available Monday to Friday between 0730 – 1600 hours.
- Patient flow back to ESSU is available Monday to Sunday between 0800-1800 hours.
- On call Clinical Toxicology advice remains 24hrs a day. See AMCOM for details.

Patient transfer from ICU to ESSU to occur Monday to Sunday between 0800 – 1800 hours.

4. Patient flow

Refer also to the Toxicology patient flow chart in [Appendix 1](#).

4.1. Triage

- 4.1.1. Risk assessment at initial ED presentation identifies that a brief period of ICU management for complication of a toxicological presentation is indicated, for example:
 - the patient requires intubation
 - the patient requires inotropic support
 - the patient requires a central line or arterial line
- 4.1.2. Patient's name is referred to both the ED ANUM and ED Medical Coordinator flagging patient to return to ESSU once extubated and not requiring ICU management
- 4.1.3. Patient name is placed on EXPECT list and EBM (Enterprise Bed Management) bed slip under Clinical Toxicology (Admitting Consultant details on AMCOM)

4.2. ICU transfer to ESSU Inclusion criteria

- Patient 4 hours post extubation with an ADDS <4

4.3. ICU transfer to ESSU Exclusion criteria

- Predicted complex significant morbidity requiring rehabilitation including severe aspiration pneumonia
- Significant behavioural disturbance not suitable for management in ESSU
- Ongoing need for inotropic support, central line, arterial line or need for IV sedation
- Patients that can be safely discharged from ICU, not requiring emergency, drug and alcohol or psychiatric input

4.4. ICU

- The Senior Registrar will alert Toxicology Registrar and ED ANUM of intent to extubate patient. This will enable ED to plan for ESSU admission after 4 hours of observation post extubation.
 - FSH Toxicology Registrar is available Monday –Friday 0730-1600. Outside of these hours alert SETS on call.

4.5. ED/Toxicology

- ANUM /Toxicology Registrar/Toxicology Physician roles:
 - Review patient in ICU to ensure they are suitable for ED. ICU will list the patient on EBM.
 - Liaise with ESSU to organise an appropriate visual bed in ESSU
 - Pull the patient on EBM with a bed ready time. The patient will be listed on the Request List view under “Clinical Toxicology” bed card.
- Weekend transfers MUST be discussed with ED ESSU Medical lead and on call SETS clinician.
- If patient has alerts for violence or is expected to have delirium, then consider Nurse special in ESSU
- If patient expresses ongoing self-harm/suicidality, consider the need for a Nurse special, and escalate as a priority to the ED Psychiatry Team for urgent review in ESSU.

Patients returning from ICU to ESSU must be approved by the ANUM, ED, ESSU Medical Lead or Toxicologist

If not deemed suitable; bed card should be transferred to General Medicine

4.6. ESSU

- 4.6.1. The ED ANUM will triage the patient on arrival to ESSU in EDIS with an ATS score of ‘A’
 - “Clock” to be stopped when patient arrives in ESSU and Medical team alerted.
- 4.6.2. Toxicology Registrar or ESSU Senior Doctor (after hours) will review the patient on arrival, facilitate appropriate ongoing supportive care, medical clearance, and ensure collaborative proactive disposition decision making, involving as appropriate:
 - Physiotherapy assessment: if concerns about aspiration pneumonia, document appropriate plan
 - Psychiatric Liaison nurse (PLN)
 - Alcohol and Drug Service

All patients will be referred on to the Mental Health Alcohol and Drug Support Line (ADSL) which results in phone follow up for two consecutive days following discharge. This also provides a safety net for patients discharging prior to review. Outcome of these referrals are fed back to the FSH AOD team and uploaded to the patient's medical record.

- Social work referral

4.6.3. Ensure disposition planning includes:

- documentation of medical clearance
- documentation of relevant Psychiatric assessment and management and safety planning on discharge
- notification of Next of Kin

5. Nursing considerations in ESSU

- ESSU Nurse to receive comprehensive bedside handover of patient from ICU Nurse.
- Ensure patient bay is cleared of potential hazards for patient and staff.
- Ensure patient and their belongings have been searched and documented.
- Ensure patients are placed in a visual bay within ESSU.
- Perform an initial set of observations (BP, HR, RR, SpO2, temp, BSL) followed by hourly for 2 hours, then as per ADDS escalation criteria on the Adult Observations and Response Chart.
- Assess and document the patient's Glasgow Coma Scale (GCS) on the Neurological Examination Chart.
- Consider Delirium and Cognitive Impairment on the Screening Tool.
- Consider Upper and Lower Limb Neurovascular observations.
- Aspiration pneumonia is relatively common post a sedative toxidrome
 - Review ICU Physio assessment and management plan
 - Encourage nursing upright and early mobilisation
- Administer patients regular and PRN medications on the WA Hospital Medication Chart – Short Stay.
- Complete Indwelling Catheter Care and Trial of Void Care Plan including bladder scans as per care plan if required.
- Complete Fluid Balance Chart.
- Consider Bowel Chart.
- Complete cardiac monitoring and ECG requests as per medical decision.

- Ensure all invasive lines are labelled appropriately, tubes are patent, secured and positioned as previously documented, and complete Peripheral Intravenous Cannula Care Plan.
- Update Falls Risk Assessment and Management Plan.
- Complete Skin Integrity Bundle.
- Complete Adult Pressure Injury Prevention and Management Plan if patient is at risk as per the Braden Pressure Injury Risk Assessment.
- Complete Wound Management Plan if required.
- Consider alcohol withdrawal and nicotine requirements in collaboration with Medical team.
- Ensure Behavioural Observation Form (BOF) is completed if patient requires a companion
- Refer to the patient's ICU admission, document patient admission to ESSU and complete all other relevant documents on the patient's digital medical record including e-diet.
- CARE checks 1 hourly unless more frequent observations ordered.
- Commence a Nursing Care Plan for each toxicology admission
- If not being admitted to a mental health facility, ensure follow up plan documented prior to discharge.

6. Psychiatry team responsibilities:

- Ensure timely liaison with ED/Toxicology staff
- Ensure documentation of psychiatric assessment.
- Formulate and document a plan for management, including a discharge safety plan. Involve support persons as indicated.
- Facilitate ongoing collaboration between Consultation Liaison Psychiatry and ED Psychiatry teams to ensure safe handover and discharge planning.

7. Transfer of care to specialty teams

- Patients not discharged directly from ESSU who require ongoing care and ward admission under a Medical or Psychiatric team will be referred to the relevant specialty i.e., Acute Medical Unit or Psychiatry
- After referral submit an EBM Bed Request:
 - Go to ESSU ward view screen and add the patient to the "Ward transfer list".
Note: An EBM Bed Request cannot be submitted on the "ED View" screen.

- The patient will be listed on the “Request List” screen on EBM.

8. Compliance/performance monitoring

Compliance against this policy will be evaluated via routine incident review processes by the appropriate designation. The Toxicology Registrar also carries out 6-monthly audits of Toxicology presentations.

9. Related policy documents

- DoH
 - Safety Planning for mental health patients – check name
- FSFHG:
 - Medication Administration (FSFH-HW-POL-0022)
 - Clinical Handover (FSFH-HW-POL-0035)
 - Clinical Documentation (FSFH-HW-POL-0039)
 - Capacity Management and Escalation (FSFH-HW-POL-0058)
 - Clinical Care of People who may be suicidal

10. Other related documents

- FSH Toxicology Model of Care

11. Related standards

- NSQHS Standards:
 - Clinical Governance
 - Comprehensive Care
 - Communicating for Safety
 - Recognising and Responding to Acute Deterioration

12. Authorisation

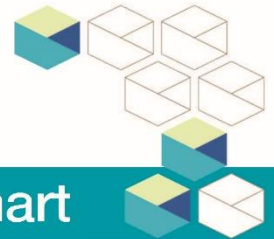
EXECUTIVE SPONSOR: Nurse Director Service 4					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	11/2021	ED ANUM, CN ED	Recognising and Responding to Acute Deterioration Committee	Service 4 SQR Committee	11/2023
2	04/2024	ED Consultant/ED ANUM	ED Clinical Governance Committee	FSFHG Policy Committee	04/2026

13. Appendices

13.1. Appendix 1: Clinical toxicology patient flowchart



Government of Western Australia
South Metropolitan Health Service
Fiona Stanley Fremantle Hospitals Group



Clinical Toxicology Patient Flowchart

Emergency to ICU to ESSU Pathway

Transfer to only occur Monday to Sunday between 0800-1800 hrs
FSH Tox Reg available Monday to Friday between 0730hrs – 1600 hrs

